

Clinical Findings

History

The typical history is of a newly acquired, slowly expanding, persistent, and often scaly lesion of the skin. Over several weeks to months, the appearance of additional nearby lesions is suggestive of local spread and supports the diagnosis of HPV infection.

Cutaneous Lesions

Warts are classified by their clinical location or morphology. The cutaneous manifestations of warts are varied, as illustrated in Figures 196-3 to 196-8.

Common warts (*verruca vulgaris*) are scaly, rough, spiny papules or nodules that can occur on any cutaneous surface. They most commonly appear as solitary or grouped lesions on the hands and fingers (see Figs. 196-3A to 196-5). These warts may also present in a **filiform** form (see Fig. 196-6) or as **cutaneous horns**.

Flat warts (*verruca plana*) are 1 to 4 mm, slightly elevated, flat-topped papules with minimal scale (see Fig. 196-8). They are most frequently found on the face, hands, and lower legs.

Plantar and palmar warts are thick, endophytic, and hyperkeratotic papules that may be painful under pressure (see Fig. 196-3B,C). After removal of the overlying keratin, punctate black dots—representing thrombosed capillaries within the papillary dermal papillae—may become visible. These are often referred to as "seeds."